

► PUBLIC HEALTH SCIENCES—COMMUNICATION SKILLS

Patient-centered interviewing techniques

Introduction	Introduce yourself and ask the patient their name and how they would like to be addressed. Address the patient by the name and pronouns given. Avoid making gender assumptions. Sit at eye level, near the patient, while facing them directly.
Agenda setting	Identify concerns and set goals by developing joint agenda between the physician and the patient.
Reflection	Actively listen and synthesize information offered by the patient, particularly with respect to primary concern(s).
Validation	Legitimize or affirm the patient's perspectives.
Recapitulation	Summarize what the patient has said so far to ensure correct interpretation.
Facilitation	Encourage the patient to speak freely without guiding responses or leading questions. Allow the patient to ask questions throughout the encounter.

Establishing rapport**PEARLS**

Partnership	Work together with patient to identify primary concerns and develop preferred solutions.
Empathy	Acknowledge the emotions displayed and demonstrate understanding of why the patient is feeling that way.
Apology	Take personal responsibility when appropriate.
Respect	Commend the patient for coming in to discuss a problem, pushing through challenging circumstances, keeping a positive attitude, or other constructive behaviors.
Legitimization	Assure patient that emotional responses are understandable or common.
Support	Reassure patient that you will work together through difficult times and offer appropriate resources.

Delivering bad news**SPIKES**

Setting	Offer in advance for the patient to bring support. Eliminate distractions, ensure privacy, and sit down with the patient to talk.
Perception	Determine the patient's understanding and expectations of the situation.
Invitation	Obtain the patient's permission to disclose the news and what level of detail is desired.
Knowledge	Share the information in small pieces without medical jargon, allowing time to process. Assess the patient's understanding.
Emotions	Acknowledge the patient's emotions, and provide opportunity to express them. Listen and offer empathetic responses.
Strategy	If the patient feels ready, discuss treatment options and goals of care. Offer an agenda for the next appointment. Giving control to the patient may be empowering. Ask how they feel a problem might be solved and what they would like to do about the plan of action.

Gender- and sexuality-inclusive history taking

Avoid making assumptions about sexual orientation, gender identity, gender expression, and behavior (eg, a patient who identifies as heterosexual may engage in same-sex sexual activity). Use gender-neutral terms when referring to the patient or the patient's family (eg, "partner" rather than "husband" or "wife") upon first meeting the patient until the patient instructs otherwise or uses specific pronouns. A patient's assigned sex at birth and gender identity may differ. Do not bring up gender or sexuality if it is not relevant to the visit (eg, a gender-nonconforming patient seeking care for a hand laceration).

Consider stating what pronouns you use when you introduce yourself (eg, "I'm Dr. Smith, and I use she/her pronouns") and asking patients how they would like to be addressed. Also consider ways of being inclusive (eg, ensuring correct name and pronouns are in the electronic medical record).

Reassure them about the confidentiality of their visits and be sensitive to the fact that patients may not be open about their sexual orientation or gender identity to others in their life.

Remember: trust is built over time, and listening to and learning from patients about how they would like to approach the topics discussed above is key.

Cultural formulation interview

Identify the problem through the patient's perspective. Ask the patient to describe the problem in their own words, or how the patient would describe the problem to their family and friends.

Identify cultural perceptions of factors leading to a problem. Ask the patient to explain why they think they are experiencing their problem.

Identify how the patient's background influences their problem. Ask the patient about what makes their problem better or worse. Investigate roles of family, community, and spirituality.

Identify how culture may impact current and future interventions. Ask the patient if they have any concerns or suggestions about the current plan of treatment. If they do not want to follow medical advice, investigate if there is a way to combine their plans with the standard medical regimen.

Identify possible barriers to care based on culture. Ask the patient if there is anything that would prevent them from seeking care in a standard medical institution. Probe for explanations and what may increase the chance of maintaining a good patient-physician relationship.

Motivational interviewing

Counseling technique to facilitate behavior modification by helping patients resolve ambivalence about change. Useful for many conditions (eg, nicotine dependence, obesity). Helpful when patient has some desire to change, but it does not require that the patient be committed to making the change. May involve asking patients to examine how their behavior interferes with their life or why they might want to change it. Assess barriers (eg, food access, untreated trauma) that may make behavior change difficult.

Assessing a patient's readiness for change is also important for guiding physician-suggested goals. These goals should be **S**pecific, **M**easurable, **A**chievable, **R**elevant, and **T**ime bound (**SMART**).

Trauma-informed care

Patients with history of psychological trauma should receive thorough behavioral health screenings. Regularly assess mood, substance use, social supports, and suicide risk.

Focus assessments on trauma-related symptoms that interfere with social and occupational function. Always be empathetic. Do not ask invasive questions requiring the patient to describe trauma in detail. Ask permission prior to discussion.

Before the physical exam, reassure patients that they may signal to end it immediately if they experience too much physical or emotional discomfort. Offer the presence of additional staff for support. Psychological counseling may be indicated. Follow-up counseling is offered (or advised) as appropriate. Remember **4 R**'s: **R**ealize, **R**ecognize, **R**espond, **R**esist retraumatization.

Challenging patient and ethical scenarios

The most appropriate response is usually one that is **p**atient-centered, open-ended, and empathetic; acknowledges the **o**bstacles in care; and **v**alidates emotions. It often honors one or more of the principles of autonomy, beneficence, nonmaleficence, and justice. Appropriate responses are respectful of patients and other members of the healthcare team. Consider the patient's **p**oint **o**f **v**iew.

SITUATION	APPROPRIATE RESPONSE
Patient does not follow the medical plan.	Determine whether there are financial, logistical, or other obstacles preventing the patient's adherence. Do not coerce the patient into adhering or refer the patient to another physician. Schedule regular follow-up visits to track patient progress.
Patient desires an unnecessary procedure.	Attempt to understand why the patient wants the procedure and address underlying concerns. Do not refuse to see the patient or refer to another physician. Avoid performing unnecessary procedures.
Patient has difficulty taking medications.	Determine what factors are involved in the patient's difficulties. If comprehension or memory are issues, use techniques such as providing written instructions, using the teach-back method, or simplifying treatment regimens.
Family members ask for information about patient's prognosis.	Avoid discussing issues with relatives without the patient's permission.
A patient's family member asks you not to disclose the results of a test if the prognosis is poor because the patient will be "unable to handle it."	Explore why the family member believes this would be detrimental, including possible cultural factors. Explain that if the patient would like to know information concerning care, it will not be withheld. However, if you believe the patient might seriously harm self or others if informed, you may invoke therapeutic privilege and withhold the information.
A 17-year-old is pregnant and requests an abortion.	Many states require parental notification or consent for minors for an abortion. Unless there are specific medical risks associated with pregnancy, a physician should not sway the patient's decision for, or against, an elective abortion (regardless of patient's age or fetal condition). Discuss options for terminating the pregnancy and refer to abortion care, if needed.
A 15-year-old is pregnant and wants to raise the child. The patient's parents want you to tell the patient to give the child up for adoption.	The patient retains the right to make decisions regarding the child, even if the patient's parents disagree. Provide information to the teenager about the practical aspects of caring for a baby. Discuss options for terminating the pregnancy, if requested. Encourage discussion between the patient and parents to reach the best decision.
A terminally ill patient requests physician-assisted dying.	The overwhelming majority of states prohibit most forms of physician-assisted dying. Physicians may, however, prescribe medically appropriate analgesics even if they potentially shorten the patient's life.
Patient is suicidal.	Assess the seriousness of the threat. If patient is actively suicidal with a plan, suggest remaining in the hospital voluntarily; patient may be hospitalized involuntarily if needed.
Patient states that you are attractive and asks if you would go on a date.	Use a chaperone if necessary. Romantic relationships with patients are never appropriate. Set firm professional boundaries with direct communication. Transition care to another physician if necessary.
A woman who had a mastectomy says she now feels "ugly."	Find out why the patient feels this way. Do not offer falsely reassuring statements (eg, "You still look good").
Patient is angry about the long time spent in the waiting room.	Acknowledge the patient's anger, but do not take a patient's anger personally. Thank the patient for being patient and apologize for any inconvenience. Stay away from efforts to explain the delay.
Patient is upset with treatment received from another physician.	Suggest that the patient speak directly to that physician regarding the concern. If the problem is with a member of the office staff, reassure the patient you will speak to that person.

Challenging patient and ethical scenarios (continued)

SITUATION	APPROPRIATE RESPONSE
An invasive test is performed on the wrong patient.	Regardless of the outcome, a physician is ethically obligated to inform a patient that a mistake has been made.
A patient requires a treatment not covered by insurance.	Discuss all treatment options with patients, even if some are not covered by their insurance companies. Inform patient of financial assistance programs.
A 7-year-old boy loses a sister to cancer and now feels responsible.	At ages 5–7, children begin to understand that death is permanent, all life functions end completely at death, and everything that is alive eventually dies. Provide a direct, concrete description of his sister's death. Avoid clichés and euphemisms. Reassure the boy that he is not responsible. Identify and normalize fears and feelings. Encourage play and healthy coping behaviors (eg, remembering her in his own way).
Patient is victim of intimate partner violence.	Ask if patient is safe and help devise an emergency plan if there isn't one. Ask patient direct, open-ended questions about exam findings and summarize patient's answers back to them. Ask if patient has any questions. Do not necessarily pressure patient to leave a partner or disclose the incident to the authorities (unless required by state law).
Patient wants to try alternative or holistic medicine.	Explore any underlying reasons with the patient in a supportive, nonjudgmental manner. Advise the patient of known benefits and risks of treatment, including adverse effects, contraindications, and medication interactions. Consider referral to an appropriate complementary or alternative medicine provider.
Physician colleague presents to work impaired.	This presents a potential risk to patient safety. You have an ethical and usually a legal obligation to report impaired colleagues so they can cease patient care and receive appropriate assistance in a timely manner. Seek guidance in reporting as procedures and applicable law vary by institution and state.
Patient's family insists on maintaining life support after brain death has occurred, citing patient's movements when touched.	Gently explain to family that there is no chance of recovery, and that brain death is equivalent to death. Movement is due to spinal arc reflex and is not voluntary. Bring case to appropriate ethics board regarding futility of care and withdrawal of life support.
A pharmaceutical company offers you a sponsorship in exchange for advertising its new drug.	Reject this offer. Generally, decline gifts and sponsorships to avoid any conflict of interest. The AMA Code of Ethics does make exceptions for gifts directly benefitting patients; special funding for medical education of students, residents, fellows; grants whose recipients are chosen by independent institutional criteria; and funds that are distributed without attribution to sponsors.
Patient requests a nonemergent procedure that is against your personal or religious beliefs.	Provide accurate and unbiased information so patients can make an informed decision. In a neutral, nonjudgmental manner, explain to the patient that you do not perform the procedure but offer to refer to another physician.
Mother and 15-year-old daughter are unresponsive and bleeding heavily, but father refuses transfusion because they are Jehovah's Witnesses.	Transfuse daughter, but do not transfuse mother. Emergent care can be refused by the healthcare proxy for an adult, particularly when patient preferences are known or reasonably inferred, but not for a minor based solely on faith.
A dependent patient presents with injuries inconsistent with caretaker's story.	Document detailed history and physical. If possible and appropriate, interview the patient alone. Provide any necessary medical care. If suspicion remains, contact the appropriate agencies or authorities (eg, child or adult protective services) for an evaluation. Inform the caretaker of your obligation to report. Physicians are required by law to report any reasonable suspicion of abuse, neglect, or endangerment.
A pediatrician recommends standard vaccinations for a patient, but the child's parent refuses.	Address any concerns the parent has. Explain the risks and benefits of vaccinations and why they are recommended. Do not administer routine vaccinations without the parent's consent.

**Communicating
with patients with
disabilities**

Patients may identify with person-first (ie, “a person with a disability”) or identity-first (ie, “a disabled person”) language. Ask patients what terms they use.

Under most circumstances, talk directly to the patient. Do not assume that nonverbal patients do not understand. Accompanying caregivers can add information to any discussion as needed. Ask if assistance is desired rather than assuming the patient cannot do something alone. Most people, including people with disabilities, value their independence.

For patients with speech difficulties, provide extra time for the interview. If their speech is difficult to understand, consider asking them to write down a few words or ask them to rephrase their sentence. Repeat what they said to ensure you understood it correctly.

For patients with a cognitive impairment, use concrete, specific language. Ask simple, direct questions. Eliminate background noise and distractions. Do not assume the patient can read. Adjust to how the patient understands best (eg, use hand gestures or ask them to demonstrate a task).

Ask patients who are deaf or hard of hearing their preferred mode of communication. Use light touch or waving to get their attention. For patients who prefer to speak and lipread, eliminate background noise, face the patient, and do not change your mode of speaking. Consider using an interpreter when necessary.

As with other parts of a medical history, do not bring up a disability if it is not relevant to a visit (eg, a patient in a wheelchair with an ear infection). Do not skip relevant parts of the physical exam even if the disability makes the exam challenging.

Use of interpreters

Visits with a patient who speaks little English should utilize a professionally trained medical interpreter unless the physician is conversationally fluent in the patient’s preferred language. If an interpreter is unavailable in person, interpretation services may be provided by telephone or video call. If the patient prefers to utilize a family member, this should be recorded in the chart.

Do not assume that a patient is a poor English speaker because of name, skin tone, or accent. Ask the patient what language is preferred.

The physician should make eye contact with the patient and speak to them directly, without use of third-person statements such as “tell him.”

Allow extra time for the interview, and ask one question at a time.

For in-person spoken language interpretation, the interpreter should ideally be next to or slightly behind the patient. For sign language interpretation, the interpreter should be next to or slightly behind the physician.

In cases of emergency, facilitate communication by any tools available (eg, friends, family, sketches, interpreter apps) even though they do not comprise standard procedure otherwise.